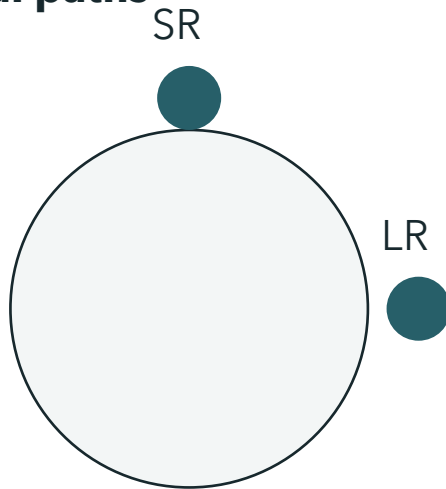


Strabismus operation selection and complications

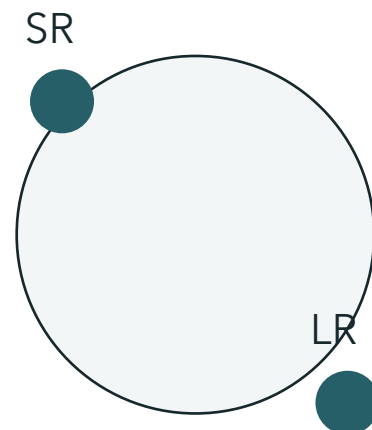
Part 2 | High myopia can alter the muscle paths

The mechanical vector can explain apparent weakness.

Usual paths



Heavy eye



Superior is up; temporal is right in each unoriented coronal schematic.

Original coronal sketch: LR inferior and SR medial in heavy eye syndrome.

High axial myopia with esotropia, hypotropia and restricted abduction or elevation should prompt assessment of globe-muscle geometry. MRI can distinguish displacement from an isolated nerve palsy or age-related pulley change. Explain why a conventional horizontal dosage may not correct the underlying path: operation selection should address the demonstrated mechanical relationship.

Heavy versus sagging eye

A similar distance deviation does not imply the same operation.

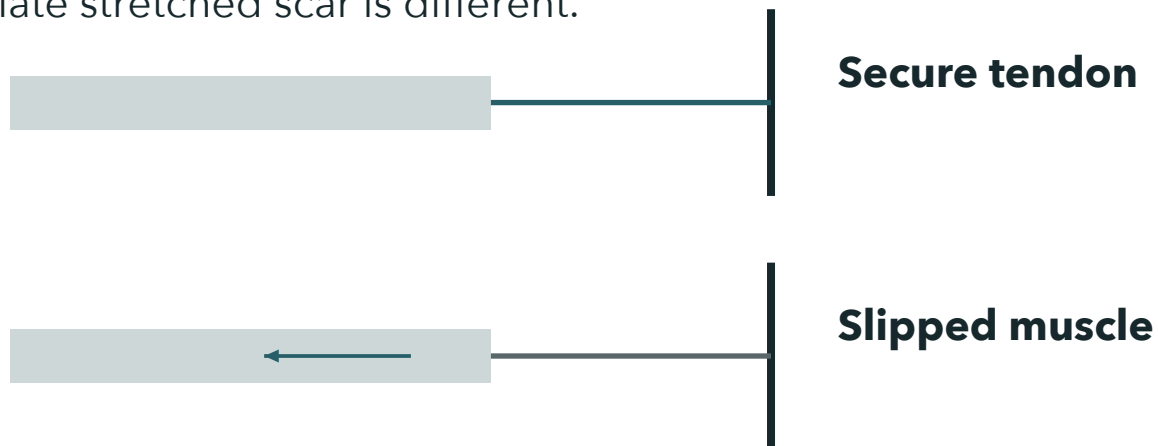
Finding	Implication
Heavy eye	Elongated globe displaces SR and LR paths
Sagging eye	Connective-tissue degeneration alters pulley support
MRI question	Is the globe outside the normal muscle cone?
Operative inference	Select geometry-directed rather than label-only correction

Myopia alone cannot establish heavy eye syndrome. Integrate the clinical restriction, vertical component and imaging geometry. In appropriate heavy-eye anatomy, SR-LR union or loop myopexy can restore the muscle relationship, with additional procedures selected for contracture. Do not apply that operation automatically to every older myopic patient with distance esotropia.

New marked underaction after surgery

Early attachment failure differs from a late stretched scar.

Early marked underaction requires attachment assessment; late stretched scar is different.

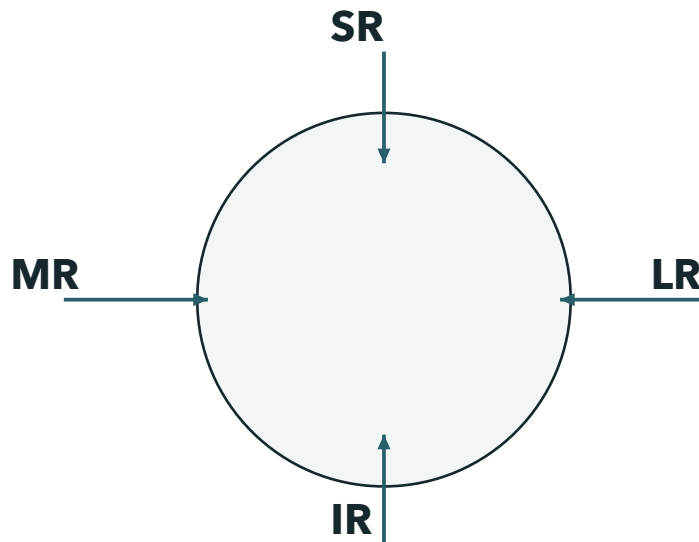


Original tendon sketch: secure attachment versus muscle retraction within a retained capsule.

Immediate large exotropia and severe loss of adduction after medial-rectus surgery raise concern for a slipped or lost muscle. Review the operative findings and seek prompt expert assessment; do not wait months under a routine adaptation explanation. The mechanical problem is failure of force transmission, and timely exploration can identify a recoverable muscle.

Anterior segment ischaemia: prevent loss of inflow

More rectus surgery can increase risk in a vulnerable eye.



Original arterial relationship: rectus-associated inflow contributes to anterior perfusion.

Assess vascular disease, previous rectus operations and buckle history when planning multiple disinsertions. Staging or suitable vessel-sparing approaches can reduce risk; a smaller conjunctival wound does not preserve an artery cut with the tendon. Explain why the safest correction may require a different sequence or technique, especially when three muscles in one eye are proposed.

Postoperative diplopia: define the remaining problem

A final primary-position angle is not the whole functional outcome.

Finding	Implication
Comitance and gaze range	Check where binocular vision is possible
Attachment or restriction	Investigate a disproportionate deficit
Sensory context	Assess fusion potential and optical correction
Further surgery	Balance benefit with scarring and perfusion risk

Compare the postoperative pattern with the original goal and expected adaptation. A mechanical complication needs a different response from residual small-angle diplopia or restricted fusion. State which additional measurement changes the next decision. Repeat surgery should target a demonstrated problem rather than simply pursue a numerical alignment in one gaze.